Non-surgical Prophylaxis

<table>
<tr><th>CHAPTER 4.14</th><th colspan="3">NON-SURGICAL PROPHYLAXIS</th></tr>
<tr><th rowspan="2">Conditions and Likely Organisms</th><th colspan="2">Suggested Treatment</th><th rowspan="2">Notes</th></tr>
<tr><th>Recommended</th><th>Alternative</th></tr>
<tr><td colspan="4">Respiratory System Infections Prophylaxis</td></tr>
<tr><td colspan="4">Cystic Fibrosis Prophylaxis</td></tr>
<tr><td>Oral antibiotic prophylaxis</td><td>Cloxacillin:<table><tr><th>Weight</th><th>Dose</th></tr><tr><td><10kg</td><td>125 mg q12H</td></tr><tr><td>10-15 kg</td><td>250 mg q12H</td></tr><tr><td>>15kg</td><td>25 mg/kg q12H</td></tr></table></td><td></td><td>• The aim of prophylactic Cloxacillin is to reduce Staphylococcus aureus infection and subsequent inflammation during the time of lung development when the lung is most vulnerable. It is used in ALL children below 3 years old or in children > 3 years when there are more than 2 isolates of MSSA in a year.</td></tr>
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<tr><td></td><td>Azithromycin (Immunomodulator):<table><tr><th>Weight</th><th>Dose</th></tr><tr><td><15kg</td><td>10 mg/kg q24H (M,W,F)</td></tr><tr><td>15-40 kg</td><td>250 mg q24H (M,W,F)</td></tr><tr><td>>40kg</td><td>500 mg q24H (M,W,F)</td></tr></table>Note: M,W,F: Monday, Wednesday, Friday</td><td></td><td></td></tr>
<tr><td>Nebulized antibiotic as prophylactic treatment
Pseudomonas aeruginosa only</td><td>Nebulised Gentamicin:<table><tr><th>Age</th><th>Dose</th></tr><tr><td>< 2 years</td><td>20 mg q12H</td></tr></table></td><td></td><td>To be used for 3 months.
Bronchospasm can be reduced by:
Diluting with water</td></tr>
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<tr><td></td><td></td><td></td><td>First dose in hospital.</td></tr>
<tr><td>Nebulized antibiotic as prophylactic treatment
Pseudomonas aeruginosa
Burkholderia cepacia complex</td><td>Nebulised Amikacin:<table><tr><th>Age</th><th>Dose</th></tr><tr><td>< 1 year</td><td>62.5 mg q12H</td></tr><tr><td>1 - 5 years</td><td>125 mg q12H</td></tr><tr><td>6 - 12 years</td><td>250 mg q12H</td></tr><tr><td>> 12 years</td><td>500 mg q12H</td></tr></table>OR</td><td></td><td>To be used for 3 months.
Bronchospasm can be reduced by:
Diluting with water
Pre-dose with bronchodilator</td></tr>
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<tr><td></td><td>Nebulised Colistin:<table><tr><th>Age</th><th>Dose</th></tr><tr><td>< 2 years</td><td>500,000 units q12H</td></tr><tr><td>2 - 8 years</td><td>1,000,000 units q12H</td></tr><tr><td>> 8 years</td><td>2,000,000 units q12H</td></tr><tr><td colspan="2">1,000,000 units = 1 Mega units</td></tr></table></td><td></td><td></td></tr>
<tr><td colspan="4">Tropical Diseases Prophylaxis</td></tr>
<tr><td>Malaria
Plasmodium falciparum
Plasmodium vivax
Plasmodium ovale
Plasmodium malariae</td><td>Atovaquone / Proguanil (Malarone)*
1 paediatric tablet = Atovaquone 62.5 mg and Proguanil 25 mg

1 adult tablet = Atovaquone 250 mg and</td><td>Mefloquine*
1 tablet: 250 mg Mefloquine hydrochloride

<15 kg:
5 mg/kg weekly</td><td>• Atovaquone/Proguanil:
Start 2 days before, continue daily during exposure and 7 days after.
• Mefloquine:
Start 2 - 3 weeks before, continue weekly during exposure and 4 weeks after.</td></tr>
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CHAPTER 4.14	NON-SURGICAL PROPHYLAXIS		
Conditions and Likely Organisms	**Suggested Treatment**		**Notes**
	Recommended	**Alternative**	
	Proguanil 100 mg **5-8 kg:** 1/2 tablet q24H **>8-10 kg:** 3/4 tablet q24H **>10-20 kg:** 1 tablet q24H **>20-30 kg:** 2 tablets q24H **>30-40 kg:** 3 tablets q24H **>40 kg:** 1 adult tablet q24H	**15-19 kg:** 1/4 tablet weekly **20-30 kg:** 1/2 tablet weekly **31-45 kg:** 3/4 tablet weekly **>45 kg:** 1 tablet weekly **OR** **Doxycycline** 1.5 mg base/kg PO q24H (max 100 mg) ***DO NOT USE for those < 8 years old OR < 25 kg*** **25 - 35 kg OR 8 - 10 years old:** 50 mg **36 - 50 kg OR 11 - 13 years old:** 75 mg **>50 kg OR >14 years old:** 100 mg	• Doxycycline: Start 2 days before, continue daily during exposure and 4 weeks after. • *Atovaquone/Proguanil (Malarone) and Mefloquine tablets are not available in UMMC.

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Conditions and Likely Organisms	**Suggested Treatment**		**Notes**
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Urinary Tract Infections Prophylaxis			
Prophylaxis is indicated for: Recurrent symptomatic UTI, VUR grade III and above	Trimethoprim* 2 mg/kg/dose (max 100 mg) PO q24H **OR** Nitrofurantoin 1 mg/kg/dose (max 100 mg) PO q24H	Co-trimoxazole 2-3 mg/kg/dose of Trimethoprim PO q24H	• A child who develops an infection while on prophylaxis, treatment should be with a different antibiotic. • Check G6PD status prior to starting any of these regimen. • *Trimethoprim (as a single agent) is not available in UMMC. • Co-trimoxazole should be avoided in infants less than 8 weeks old.